INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Atezolizumab 1680 mg · every 4 weeks · lung cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00593
Version 10a

Treatment stages

01 **Atezolizumab every 4 weeks**
One immunotherapy medicine

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	10
04	Sources and further information	16

My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the oncology team immediately for new or worsening breathlessness, chest pain, persistent diarrhoea, severe tummy pain, yellow skin/eyes, much less urine, severe headache, marked weakness, confusion, unusual thirst or passing much more urine, or a spreading/blistering rash. For severe breathing difficulty, collapse, seizure or other immediately dangerous symptoms, seek emergency care. Do not wait for the next infusion.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

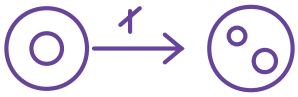
Three medicines. Different jobs.

In advanced cancer the aim is cancer control and symptom relief; after surgery the aim is to reduce recurrence risk. Confirm your exact treatment setting.

IMMUNOTHERAPY

Atezolizumab

Tecentriq



Blocks PD-L1 to support immune attack

Atezolizumab blocks PD-L1, a protein that can help cancer cells evade immune attack. Blocking PD-L1 can help immune cells attack cancer, but can also cause inflammation in healthy organs.

Immune cell → PD-L1 blocked → Cancer attacked

What to watch for

Report new breathing, bowel, skin, hormone, heart or nerve symptoms promptly; these may be immune inflammation.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your lung cancer and treatment aim

Lung settings: previously treated non-small-cell lung cancer (NSCLC); first-line PD-L1-high metastatic NSCLC; adjuvant NSCLC after surgery and platinum chemotherapy; extensive-stage small-cell lung cancer (ES-SCLC) maintenance after induction.

Confirm the setting

First-line metastatic NSCLC requires PD-L1 ≥50% tumour cells or ≥10% immune cells and no EGFR/ALK alteration. After prior chemotherapy, NSCLC has separate eligibility. Adjuvant use requires completely resected high-risk stage II–IIIA AJCC7, predominantly non-squamous cancer, PD-L1 ≥50%, no EGFR/ALK alteration and prior platinum chemotherapy. ES-SCLC use is maintenance after completed combination induction.

After induction or surgery

Adjuvant: maximum 12 months. Advanced NSCLC and ES-SCLC maintenance: until progression or unacceptable toxicity. ES-SCLC requires completed induction; this suite contains no induction chemotherapy.

Other options

Discuss other systemic treatments, targeted therapy when appropriate, clinical trials, surveillance and symptom-focused care with your team.

How atezolizumab works

This is an immunotherapy medicine.

Healthy organs can be affected

Immune inflammation can affect lungs, bowel, liver, kidneys, skin, glands, muscles, heart and nerves. It may need steroids, hospital care or long-term hormone replacement, and can start after treatment ends.

Why this treatment may be offered

Atezolizumab may help the immune system control suitable lung cancers. After surgery and chemotherapy, it can reduce recurrence risk in a selected group. In small-cell cancer it continues an induction-plus-maintenance treatment plan. These settings have different evidence and aims; trial results cannot predict one person’s outcome.

Separate lung settings and trials; group results are not personal predictions.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Induction → review → maintenance

Atezolizumab 1680 mg · every 4 weeks · lung cancer

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ONE IMMUNOTHERAPY MEDICINE

Atezolizumab every 4 weeks

C1 C2 C3 …

Adjuvant: maximum 12 months. Advanced non-small-cell lung cancer (NSCLC) and extensive-stage small-cell lung cancer (ES-SCLC) maintenance: until progression or unacceptable toxicity. ES-SCLC requires completed induction; this suite contains no induction chemotherapy.

Maintenance means ongoing treatment, not treatment without side effects. Your plan can change after discussion.

BASELINE → TREATMENT → FOLLOW-UP

Checks across your treatment

This is a relative treatment map, not a booking calendar. Ask who books each test and reviews the result. Extra tests may be needed for symptoms.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00593 v10a · official regimen

THE WHOLE TREATMENT JOURNEY

Atezolizumab 1680 mg · every 4 weeks · lung cancer: treatment and checks

Atezolizumab 1680 mg is given by intravenous infusion on Day 1 every 4 weeks (28 days). The first infusion lasts about 60 minutes; later infusions may last 30 minutes if tolerated. The visit includes checks and preparation. Adjuvant: maximum 12 months. Advanced NSCLC and ES-SCLC maintenance: until progression or unacceptable toxicity. ES-SCLC requires completed induction; this suite contains no induction chemotherapy.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First infusion	Between infusions	Course review	After treatment
Treatment NCCP 00593	Confirm exact indication	1680 mg intravenous (IV); first 60 minutes	Day 1 every 4 weeks	Adjuvant maximum 12 months; advanced/ maintenance review	Follow-up and late immune effects
Blood count, kidney, liver and glucose NCCP 00593	Baseline blood tests	Results reviewed before dosing	Before every 4-week cycle	Continue checks during holds/restarts as directed	Follow unresolved abnormalities
Thyroid and other hormones NCCP 00593 · SMPC	Baseline thyroid function; other hormones if needed	Review symptoms and hormone medicines	Thyroid every 3–6 weeks; cortisol if clinically indicated	Confirm replacement and steroid-taper plan	Continue hormone follow-up if needed
Symptoms and immune effects NCCP 00593 · SMPC	Discuss existing illnesses and warning symptoms	Tell the nurse immediately about a reaction	Report new symptoms on any day	Hold/restart decisions need clinical review	Late immune effects remain possible
Cancer response and course length NCCP 00593	Confirm the exact lung setting	Agree how benefit will be assessed	Scans on your consultant's schedule	Adjuvant up to 12 months; advanced/ maintenance individual review	Agree surveillance or next treatment

What each check is for

1

Before first dose

Before starting

Verify setting and eligibility

Ask / act: Confirm biomarkers, baseline tests, consent, prescription and alert card.

2

Course review

During treatment

Duration differs by setting

Ask / act: Document continuation, hold or stop and arrange follow-up.

3 After treatment

After the last dose

Immune effects may be delayed

Ask / **act:** Provide urgent contact and follow-up plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Appointments

STEP 2

Course length

Adjuvant treatment lasts up to 12 months. Advanced NSCLC and ES-SCLC maintenance continue until progression or unacceptable toxicity, with regular individual review.

STEP 3

Checks between visits

Blood tests are due before each infusion. Thyroid tests stay every 3–6 weeks regardless of dose schedule. Ask about travel, work, transport and whom to contact if a test or visit is delayed.

STEP 4

Home medicines and records

Keep the alert card, symptom record, current medicine list and urgent contact details accessible. Your team may prescribe steroids or hormone replacement only if needed.

STEP 5

After the final dose

Keep reporting new symptoms. Some hormone medicines and monitoring continue after the cancer treatment finishes.

Your treatment course

Appointments and total course length are separate decisions.

Pauses and changes

Your team may withhold or stop treatment for side effects. Atezolizumab is not routinely dose-reduced. Tell the team about a missed appointment; do not assume the old appointment schedule still applies.

Tests and review

Normal recent tests do not exclude new immune symptoms.

Before starting

Confirm diagnosis, treatment setting and relevant biomarkers. Blood count, kidney/liver profile, glucose, thyroid and hepatitis screening provide a baseline. Discuss existing illness and all medicines.

During treatment

Blood count, kidney/liver tests and glucose before every 4-week treatment cycle; thyroid every 3–6 weeks; other hormones when clinically indicated. Symptoms need review during and after treatment.

After stopping

Continue agreed symptom, hormone and scan follow-up. Keep your immune alert card and urgent contact details.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Infusion	 **Day 1** Day 1 · hospital infusion
Immune effects	 **Days 1–28** Any day · also after treatment ends
Energy, skin and bowels	 **Days 1–28** Timing varies · report changes promptly

Treatment setting: Lung settings: previously treated NSCLC; first-line PD-L1-high metastatic NSCLC; adjuvant NSCLC after surgery and platinum chemotherapy; ES-SCLC maintenance after induction.

Checks: Blood count, kidney/liver tests and glucose before every 4-week treatment cycle; thyroid every 3–6 weeks; other hormones when clinically indicated. Symptoms need review during and after treatment.

Duration: Adjuvant: maximum 12 months. Advanced NSCLC and ES-SCLC maintenance: until progression or unacceptable toxicity. ES-SCLC requires completed induction; this suite contains no induction chemotherapy.

> **Why these bands stay broad:** The diagram shows one reference interval; appointments, treatment duration and monitoring are individual. Immune effects may occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed “safe days”.** Immune effects can happen on any day and after treatment ends. Carry your immunotherapy alert card and tell any healthcare team that you receive atezolizumab.

Between-treatment notes

Day 1: Your infusion visit

The team reviews your blood results, symptoms and medicines. Tell the nurse straight away about itching, flushing, a tight chest, dizziness or feeling suddenly unwell during the drip. Before leaving, confirm the next tests, treatment visit and urgent contact number.

Day 2–28: Between visits

You may feel well between infusions, but immune inflammation can begin at any time. Keep a short symptom record and take medicines exactly as prescribed. Contact your team about new symptoms instead of waiting for your next appointment. Confirm thyroid checks every 3–6 weeks independently of infusion appointments.

Support around the medicines

1 

Sickness medicines

Atezolizumab alone has a minimal risk of causing sickness. Routine anti-sickness medicines or infusion premedicines are not usually needed; your team can prescribe symptom relief if required.

2

Immune alert card

Carry your alert card during treatment and afterwards. Show it when seeking urgent care; symptoms can be mistaken for an infection or an ordinary stomach upset.

3

Hormones and steroids

Some immune effects need steroids or hormone replacement. Follow the prescribed plan and do not stop steroids suddenly. Ask for help if symptoms worsen or you cannot take essential medicines.

4

Pregnancy and breastfeeding

Discuss pregnancy, fertility and breastfeeding before treatment. Use effective contraception during treatment and for at least 5 months after the last dose. Breastfeeding requires a specialist decision; contact your team promptly if pregnancy occurs.

Practical support and everyday life

Keep your team informed about what matters to you.

Energy and activity

Fatigue can affect daily activities. Balance activity with rest and ask for help if tiredness is new, marked or accompanied by dizziness, headache, thirst or weakness; it may need medical assessment.

Eating, drinking and bowels

Drink as advised and report ongoing vomiting or diarrhoea. Do not assume diarrhoea is a minor stomach upset; your team needs to check for immune inflammation and other causes.

Medicines and pregnancy

Bring a full medicine list, including supplements and steroids. Ask before starting new medicines. Use the contraception plan and discuss fertility or breastfeeding with your team.

When to get urgent help

Use the contact plan given by your oncology service.

During the drip

Tell the nurse immediately if you become itchy, flushed, breathless, dizzy, tight-chested or suddenly unwell.

Care after treatment ends

Immune inflammation can start late. Tell any healthcare team about atezolizumab even after the course has finished.

Questions worth asking

Is my treatment for metastatic cancer or to reduce recurrence after surgery?

Ask your team to write down the treatment aim. For metastatic disease, the aim is usually cancer control, symptom relief and more time. After complete surgery and platinum chemotherapy, adjuvant treatment aims to reduce the chance of recurrence.

Which tumour-test results support this plan?

For the first-line metastatic monotherapy pathway, the source requires high PD-L1 expression and no EGFR mutation or ALK translocation. The adjuvant pathway has different eligibility. Ask which results apply to your own cancer.

Who arranges blood and thyroid tests between visits?

The oncology team arranges and reviews tests. Blood count, kidney/liver function and glucose are checked before each cycle; thyroid tests are usually every 3–6 weeks. Ask whether an extra test is needed between treatment visits.

Which symptoms should I report straight away, and whom do I contact out of hours?

Report new or worsening breathing problems, persistent diarrhoea, severe abdominal pain, yellow eyes, reduced urine, severe headache, weakness, thirst or a spreading rash promptly. Keep the local oncology and out-of-hours numbers available; severe breathing difficulty, collapse, seizure or severe chest pain needs 112 / 999.

What are my alternatives if I choose not to have this treatment?

Ask about alternatives for your exact stage and tumour results, including other anticancer treatments, trials, surveillance where appropriate and symptom-focused care. Supportive care remains available whether or not you proceed.

Try explaining the plan back: Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Atezolizumab

- Immune inflammation of lungs, bowel, liver, kidneys, hormone glands, skin, heart and nervous system
- Infusion reactions
- Some effects need steroids, hospital care or long-term hormone replacement
- Effects may start after the final dose

Lung cancer treatment settings Lung settings: previously treated non-small-cell lung cancer (NSCLC); first-line PD-L1-high metastatic NSCLC; adjuvant NSCLC after surgery and platinum chemotherapy; extensive-stage small-cell lung cancer (ES-SCLC) maintenance after induction. Urothelial uses in the shared clinical protocol require their own patient discussion.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Atezolizumab 1680 mg every 4 weeks

Active medicines: atezolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis or premedication is usually required; individual rescue prescription as indicated.
- Carry immune alert card and report new symptoms immediately.
- full blood count (FBC), renal/liver profile and glucose each cycle; thyroid every 3–6 weeks.
- Effective contraception during treatment and at least 5 months after the last dose; discuss breastfeeding.
- Current NCIS v8 medical-oncology guidance reviewed. Its low/moderate/high cytotoxic default prescriptions are not applied to minimal-risk atezolizumab monotherapy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

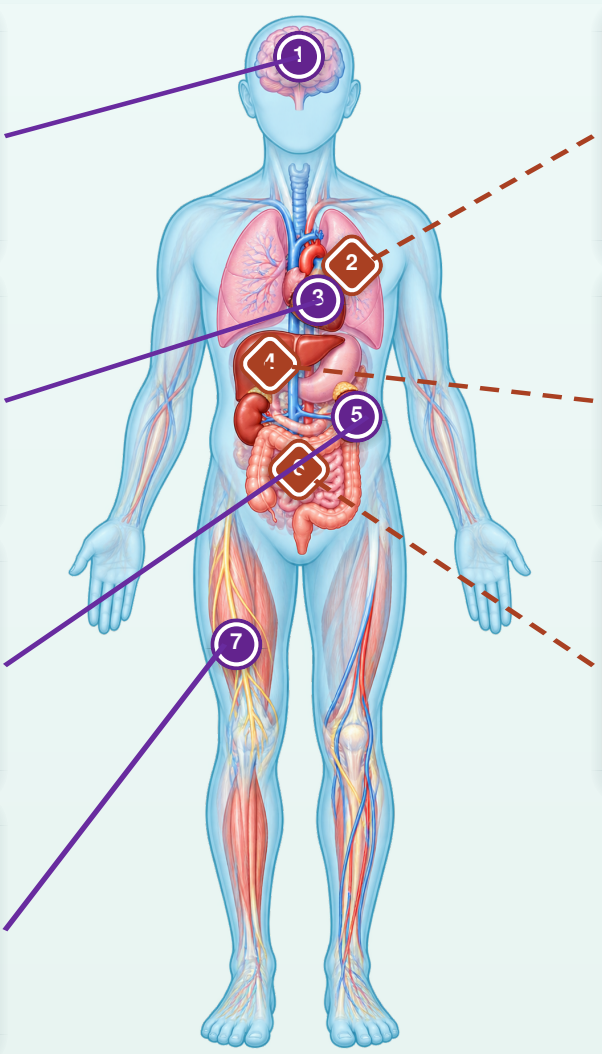
Atezolizumab 1680 mg · every 4 weeks · lung cancer

Atezolizumab 1680 mg is given by intravenous infusion on Day 1 every 4 weeks (28 days). The first infusion lasts about 60 minutes; later infusions may last 30 minutes if tolerated. The visit includes checks and preparation. Adjuvant: maximum 12 months. Advanced non-small-cell lung cancer (NSCLC) and extensive-stage small-cell lung cancer (ES-SCLC) maintenance: until progression or unacceptable toxicity. ES-SCLC requires completed induction; this suite contains no induction chemotherapy.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Hormones / nervous system

Atezolizumab

Severe headache, confusion, weakness, dizziness or marked thirst need prompt advice.

2 Lungs

Atezolizumab

New cough or breathlessness can signal inflammation or another urgent cause.

3 Heart

Atezolizumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

4 Liver

Atezolizumab

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

5 Kidneys

Atezolizumab

Report much less urine, new swelling or a change in kidney tests.

6 Bowel

Atezolizumab

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

7 Skin / muscles

Atezolizumab

Spreading rash, blisters, severe weakness or muscle pain needs prompt review.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team immediately for new or worsening breathlessness, chest pain, persistent diarrhoea, severe tummy pain, yellow skin/eyes, much less urine, severe headache, marked weakness, confusion, unusual thirst or passing much more urine, or a spreading/blistering rash. For severe breathing difficulty, collapse, seizure or other immediately dangerous symptoms, seek emergency care. Do not wait for the next infusion.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00593 v10a · official regimen**
   https://healthservice.hse.ie/documents/6399/593_v10a_Atezolizumab_1680mg_Monotherapy.pdf
2. **EMA Tecentriq product information, updated 21 July 2026**
   https://www.ema.europa.eu/en/documents/product-information/tecentriq-epar-product-information_en.pdf
3. **OAK final overall survival follow-up, Mazières 2021**
   https://pubmed.ncbi.nlm.nih.gov/33166718/
4. **IMpower110 updated exploratory analysis, Jassem 2021**
   https://pubmed.ncbi.nlm.nih.gov/34265434/
5. **IMpower010 five-year final DFS and second interim OS, Felip 2025**
   https://pubmed.ncbi.nlm.nih.gov/40446184/
6. **IMpower133 updated induction plus maintenance strategy, Liu 2021**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC8078320/
7. **HSE · chemotherapy safety and when to contact your team**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **NCCP NCIS medical-oncology antiemetic guidance v8,2025**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf